Staff Pages

Mental Health Group Therapy Worksheets for Adults · Volume 2

For staff, not for client handouts: timing and shorter routes, clinical adaptations and private handoff, who to call, and continuity and privacy.

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One Topic, Different Entry Points

For facilitators: one topic, several ways in. Nobody has to explain why they need an adaptation.

New today

Give a short explanation, a made-up example, and one manageable prompt from A, B, or C. Explain the choices, the limits of privacy, and how to get help. Don't assume they saw earlier sessions.

Seen this topic before

Offer the "On another visit" prompt, the different task in D, an unfinished plan, or a smaller version. Ask which would help. Returning members aren't unpaid co-facilitators.

Writing or attention is difficult

Use the page's spoken, pointed, drawn, or watched option. Read one prompt aloud, show an example, and check your explanation in whatever way suits the person. Arrange language or access support through the program.

Discussion takes the whole group

Acknowledge the concern, name what's left on the agenda, and ask whether to go back to practice or follow up privately. A brief round with the option to pass can help. Don't require anyone to disclose or give feedback.

Someone is quiet or finishes early

Offer watching, a made-up example, the revisit task, or thinking about a real-life barrier. Being quiet doesn't mean someone has checked out, and finishing quickly doesn't mean they've mastered it.

Tension, distress or unsafe content appears

Pause the exercise, settle the group, and follow your program's procedures. Take individual safety or means-access conversations somewhere private. An urgent concern always comes before the agenda.

Prepare, Deliver, Hand Off

For staff: not a client handout

A planning form for the facilitator. Clinical notes go in your program's record system, not here.

Choose the work

Date / group: _____ Code / page: _____ Purpose linked to care plans: _____

Prepare a brief introduction

One key idea: _____ Fictional demonstration / materials needed: _____

Prepare participation routes

Newcomer prompt: _____ Returning route: _____ Format or access adaptations: _____

Plan the frame and time

Opening / practice / discussion / close / breaks: _____ What should be handled privately or escalated under policy? _____

Notice what actually happened

What I delivered and adapted: _____ What I saw: _____ What the member said, if anything: _____ An unfinished sheet tells you nothing about mastery, risk, or consent.

Close and hand off

Agreed follow-up / who / by when: _____ What the next facilitator needs to know (through the proper handoff): _____

For private clinical handoffs, see page 123. A handoff is done when the receiver confirms what they're taking on. A client worksheet never carries the whole handoff.

Clinical Adaptations and Private Handoff

For staff: not a client handout

For the professional. Match the practice to the person's care plan and to your own competence. These are prompts, not treatment protocols for specific conditions.

Psychosis and unusual experiences

Use concrete, present-focused tasks and qualified individual clinical support. A group thought record is not a vehicle for arguing about delusions or voices. The inner critic metaphor describes self-critical thinking; hearing voices requires its own clinical conversation. Do not infer lack of motivation from a different way of communicating. [20]

Mood elevation and reduced sleep

Unusually little sleep with increased energy, agitation, impulsivity or elevated mood calls for prompt clinical review. Reduce stimulation and task demands as clinically appropriate; do not substitute an activation goal, relaxation exercise or routine log for assessment. [21]

Neurodivergence language and sensory access

Offer one concrete instruction at a time, visual or spoken choices, a demonstration and breaks. Explain metaphors literally or omit them. Ask about sensory comfort, pain, vision, hearing, movement and communication needs. Arrange professional interpretation when needed, with appropriate privacy. Do not assume eye contact, fast writing or public speech establishes engagement. [18,19]

Culture resources and relationships

Ask which values, family roles, faith practices and communication styles the person chooses. Consider racism, disability, housing, food access, work and caregiving. A boundary script or small task cannot solve every structural barrier. Preserve the choice not to disclose. [16]

When staff change

Use your program's private clinical system. Hand off the session code, what you delivered and adapted, any current clinical concerns, where the care or safety plan is kept, pending referrals and contacts, and the next action with a named person and a time. Confirm that the receiver has what they need and accepts the next step. A client's worksheet or Passport is never the clinical record. [16,17]

Staff Page: Who to Call

For staff. Fill in and check with your clinical lead. This isn't a client safety plan.

For each situation, write the role to call, a working number, what you do first, who stays with the group, and where the handoff goes. Test the numbers. No client names on this page.

Medical emergency, possible overdose, breathing or consciousness concern

Suicide or self-harm concern: who assesses, emergency route, who stays with the group

Threats or violence: protection, security or emergency route, where it's recorded

Intoxication, withdrawal, or possible medication reaction

Acute psychosis, mania, severe disorganization, or an eating-related medical concern

A worksheet is never the response to an acute medical or psychiatric concern. Start your local response right away; don't wait an arbitrary time for a callback.

Staff Page: Continuity and Privacy

For staff. Fill in and check with your clinical lead. This isn't a client safety plan.

Video groups: confirmed location, callback number, local emergency service, plan if the connection drops

Safeguarding, mandated reporting, or duty to protect: who to consult

Private space and who stays with the group; leaving and rejoining

Records: who owns them, which system, who has access, how long they're kept, how they're destroyed

Incidents: who is notified, who reviews, by when

The sender states the concern, what's been done, what's uncertain, the next step, and the backup plan. The receiver confirms they understand and what they're taking on. Write down who accepted what, and when. A sent message isn't a handoff.

Explain confidentiality limits and your reporting duties before people take part. Ask members to protect each other's privacy, and be honest that staff can't guarantee it.

Shorter Groups

For skills, psychoeducation, and PHP groups, in minutes for opening / demonstration / one complete practice / debrief / close: 30 = 4/4/12/5/5; 45 = 6/9/18/7/5; 50 = 7/10/20/8/5; 60 = 8/12/25/10/5; 90 = 12/18/35/15/10. When time is short, cut explanation and extra examples before you cut the practice or the close. Safety checks and access adaptations apply at every length. Process groups use their own plan.

Choose what serves this session

Use one task instead of rushing through the whole page. People can speak, point, draw, watch, pass, or ask for another format. Keep time for discussion and a proper close.

When a page no longer fits

Check privately what isn't working. Change the example, the task, the sensory demands, or the amount of work. Take clinical concerns to the team; more worksheet time isn't the answer.